A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

27 May 2026

Dr Rebecca Li
East Brunswick Medical Centre

Dear Dr Li,

Re: Ms Alana Moretti
DOB: 19/09/2001 Phone: 0406 560 759
Address: 4 Keane Street, COBURG, VIC, 3058
Email: morettialana1@gmail.com

Summary

- **Post-infectious functional bowel symptoms following confirmed Cryptosporidium infection.**
- **Normal investigations including faecal calprotectin, repeat stool studies, and abdominal ultrasound.**
- **Trial of conservative management and as-needed nizatidine for dyspepsia.**
- **Threshold for gastroscopy and colonoscopy if symptoms escalate.**
- **No action required by you.**

I had the pleasure of reviewing Allana Moretti regarding her ongoing gastrointestinal concerns. As you are aware, she experienced an episode of diarrhoea in December following a holiday, which was subsequently confirmed to be Cryptosporidium, likely contracted through her work in childcare.

While the acute infection settled, Allana describes recurrent episodes of loose, yellow stools that are occasionally difficult to flush. These symptoms flared significantly in March, accompanied by abdominal cramping and a temporary reduction in appetite. Currently, she experiences these changes for three to four days at a time, although her frequency remains stable at once or twice daily. Importantly, there has been no history of haematochezia or mucus. She also noted a new onset of indigestion and epigastric burning since the initial infection.

On examination, Allana was comfortable. Her abdomen was soft and non-tender, and I could not appreciate any masses or organomegaly. I have reviewed her investigations, which are very reassuring. Her repeat stool studies were negative for parasites, her faecal calprotectin was normal, and her liver function tests, pancreatic enzymes, and abdominal ultrasound were all unremarkable.

Impression and Plan

I suspect Allana is experiencing a post-infectious functional gastrointestinal disorder following her cryptosporidiosis. The probability of serious underlying pathology such as inflammatory bowel disease or a pancreatic issue is quite low given her normal inflammatory markers and imaging.

We discussed the options of proceeding directly to a gastroscopy and colonoscopy versus a period of clinical observation. Allana is keen to avoid invasive procedures if possible. I have advised her that we can safely monitor her progress, as it is likely these post-infectious symptoms will continue to improve over time. I have

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suggested she remain moderate with her intake of alcohol and fatty foods, as these appear to be occasional triggers.

For her dyspeptic symptoms, I have provided a prescription for nizatidine to be used on an "as-required" basis, which may provide more rapid relief than her current occasional use of esomeprazole. I have told Allana that if she experiences another severe flare similar to the March episode, or if she develops any red-flag symptoms, we should proceed directly to gastroscopy and colonoscopy to definitively rule out conditions such as peptic ulcer disease or atypical Crohn's disease. I will review her again only if her symptoms fail to resolve or if they escalate in severity.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

All appointments and enquires
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Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065